Talking to Pain

MI + CBT: a hybrid toolkit for chronic pain · Dr. Jason Young

MI is for the patient who won't

CBT is for the patient who can't

THE 2-QUESTION TRIAGE

1 · Are they willing?
Listen for: "yes, but" · blame · hopelessness · sustain talk

No → **MI MODE**
Reflect. Roll with it. Rulers. Evoke. Do **not** teach skills yet.

Yes ↓

2 · Are they able?
Listen for: crashing · catastrophizing · avoiding · not sleeping

No → **CBT MODE**
ONE skill, taught via Elicit–Provide–Elicit. Micro-homework, made SMART.

Yes ↓

Willing and able → reinforce, progress the plan, affirm the identity: "You're becoming someone who manages this."

Neither? MI first, always. Motivation before mechanics.

Switching cues: "Yes, but…" or silence → back to MI (you drifted into lecture) · "So what do I actually *do*?" or change talk peaks → deliver one CBT skill now · homework not done → "What got in the way?" — never "be more compliant"

SCREEN, DON'T GUESS

STarT Back (low back pain, 9 items) — low / medium / **high** risk. Psychosocial subscale ≥4 = high risk → **MI + CBT move to the front.**

PHQ-4 on every intake — 2 depression + 2 anxiety items. Score ≥3 on either pair = yellow flag.

PHQ-2 ≥3 → PHQ-9 — PHQ-9 ≥10 = a referral conversation. **Item 9 positive = same-day action.**

High psychosocial load ≠ "refer out." It means: the conversation IS the treatment.

MI — the tool for WILLING

The spirit: partnership · acceptance · compassion · evocation. Not manipulation — help them find *their* path, not yours.

The righting reflex: your urge to fix & lecture. It reliably produces "yes, but…" Pushback = wrong tool, not a bad patient.

OARS: Open questions ("What's this pain been costing you?") · **A**ffirmations (name a strength you saw) · **R**eflections (aim 2 per question; a statement, voice down) · **S**ummaries (hand their change talk back).

Two voices: change talk (DARN-CAT: desire, ability, reasons, need → commitment, activation, steps) vs. sustain talk. Water the change talk; never argue with "I've tried everything."

Rulers: "0–10, how important…? Why a 5 and not a 2?" The follow-up IS the technique. Importance ↔ willing; confidence ↔ able.

EPE: "What do you already know about…?" → ask permission, share ONE piece → "What do you make of that?"

CBT — the tool for ABLE (five skills)

1 · Reframe: hurt ≠ harm. Chronic pain = a smoke alarm gone sensitive — real noise, no fire. Deliver inside EPE, never as a lecture.

2 · Pacing: activity by the clock, not the pain. Stop *before* the flare; start below bad-day capacity; raise it a notch weekly. Boom–bust is the enemy.

3 · Catch–check–change: name the hot thought ("lifting will blow a disc") → test it against their own evidence → a believable alternative: "Sore but safe."

4 · Relaxation: 4-in / 6-out breathing on the table (the long exhale is the lever) · box breathing · PMR · imagery. Prescribe like exercise: dose, log, review.

5 · SMART goals: Specific · Measurable · Achievable · Relevant · Time-bound. The patient supplies the goal; SMART supplies the shape; your next visit is the review.

Say the name: "This is CBT — one of the best-studied tools for people who want change." Patients who know the word can find their own resources.

SHRINK THE DOSE, NOT THE METHOD

10-minute visit
1 open question while assessing → 1–2 reflections during hands-on care → 1 micro-skill on the table → "What's the one thing you'll try before Thursday?"

25–30 minutes
5′ MI (rulers, locate readiness) → 10′ teach ONE skill, practice in-room → 5′ "what could get in the way?" → 5′ summary in their words + written SMART homework

60-minute consult
Engage (their story, uninterrupted) → Focus ("pain, sleep, activity, stress — where do we start?") → Evoke (rulers, change talk) → Plan (one skill + graded walking + commitment)

Minimum effective dose, every encounter: one MI move + one CBT move. Twelve 10-minute talks ≈ two therapy sessions — the care plan IS the protocol.

RESOURCES

For patients — relaxation & skills: VA CBT-CP patient handouts (breathing, PMR, imagery scripts — free online) · Mindfulness Coach app (VA, free) · Insight Timer (free guided sessions) · CBT-i Coach app (pain-disrupted sleep) · graded walking log + SMART goal card (from today's handouts)

For clinicians: Miller & Rollnick, *Motivational Interviewing*, 4th ed. · Naar & Safren, *Motivational Interviewing and CBT* · VA *CBT-CP Therapist Manual* (free PDF) · Williams et al. 2020, Cochrane CD007407 · Hawk et al. 2020, chiropractic chronic MSK pain guideline · Keele STarT Back tool & scoring (free)

REFER WHEN

- PHQ-9 ≥10, or moderate–severe anxiety
- Any positive on PHQ-9 item 9 — same day
- PTSD / trauma beyond your training
- Substance-use concern
- Still stuck despite your best MI + CBT

A warm handoff ("I know someone excellent for exactly this") keeps the patient — and the relationship.

Willing? Able? Two questions — every visit, every patient.

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Materials + CE list